Reproductive Health

Health – State of physical, emotional, social, Behavioral well being

Reproductive health – Person is reproductively healthy in all terms

Family Planning Program – 1951

Reproductive and Health Care Programs (RCH)

Aim → Create reproductive health awareness.

Strategy →
1. Audio-visual advertisments for awareness.
2. Sex education in school.
3. Awareness about sex crimes.
4. Equal rights to males and females.
5. Menstrual hygiene awareness.
6. Contraceptive awareness

Amniocentesis test used to detect genetic abnormalities

Banned in INDIA → Female foeticide.

SAHELI
- Oral contraceptive only for females
- Non Steriodal Oral Contraceptive.
- By → CDRI lucknow
- Once a week pill
- Only for female

Eg.



Population No. of similar species in a particular area at a particular time.

Demography Study of population.

Mortality Rate No of deaths/1000 individual/year.

Natality Rate No. of Birth/1000 individual/year

Migration Movement of individual from place to place.

Infant Mortality Rate No. of Infants death/1000/Year

Maternal Mortality Rate No. of mothers death/1000/Year

Birth Control Methods/Contraceptive Methods

- to avoid unwanted pregnancies.

Ideal Contraceptive –
- user friendly
- Effective
- Reversible
- cost effective
- least/no side effects
- Easily available

Contraceptive

Temporary Method
- Natural method
- Barrier method
- IUDs
- Hormonal Method

Permanent Methods
- Surgery
- Vasectomy
- Tubectomy

Natural Methods –
- Avoiding chance of ovum - sperm meeting
- No side effects

a) **Periodic abstinence / cyclic method / calender method**
- avoiding intercourse from 10-17th day of cycle

b) **Coitus interruptus / withdrawal method**

. Penis withdraw from the vagina just before the ejaculation / Insemination

c) **Lactation ammenorrhea**

Duration – 6 Months after parturition

Barrier Method

Sperm either killed/phagocytosis or stopped to reach ova

Physical Method
- Condom
- Diaphragm, vaults, Cervical caps

Chemical Method
- Cream, Jellies etc

Condom
– Made up of rubber/latex.
- Protects from STD

Nirodh – used in INDIA
- Non-reuseable



Figure 3.1(a) Condom for male

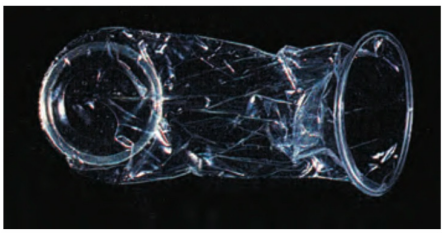
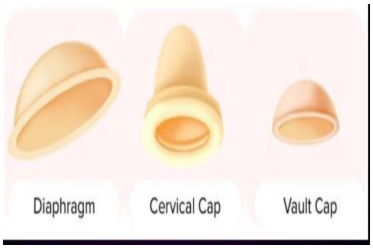


Figure 3.1(b) Condom for female

Diaphragms/Vaults/Cervical Cap
- for females only

- Cover cervix/cervical canal
- Reuseable
- Prevents entry of sperm to ova



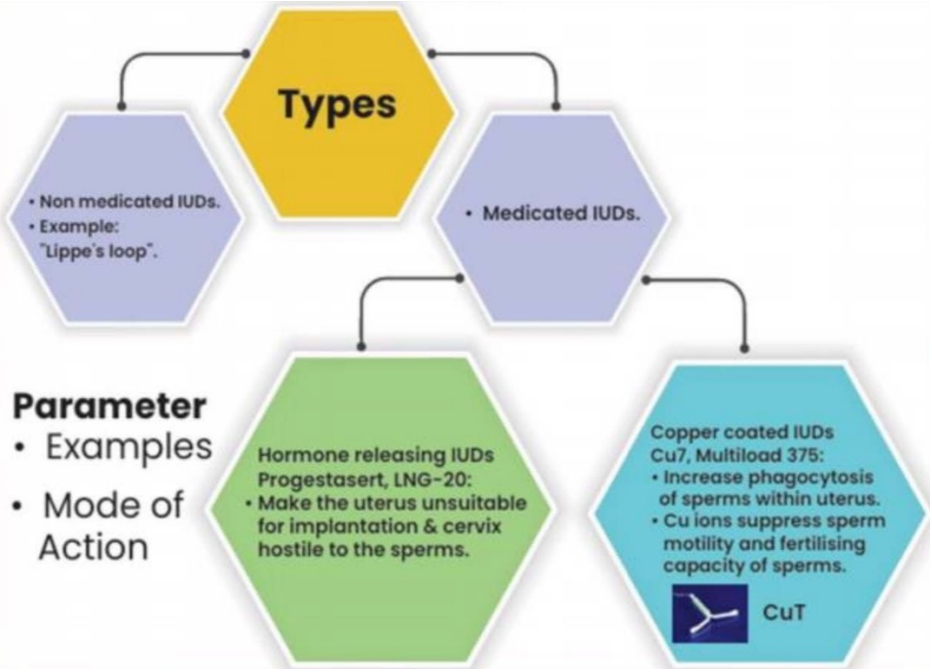
Chemical Method
- Creams, foams, Jellies
- Spermicidal – cause phagocytosis of sperm
- used with physical barrier
- Very effective

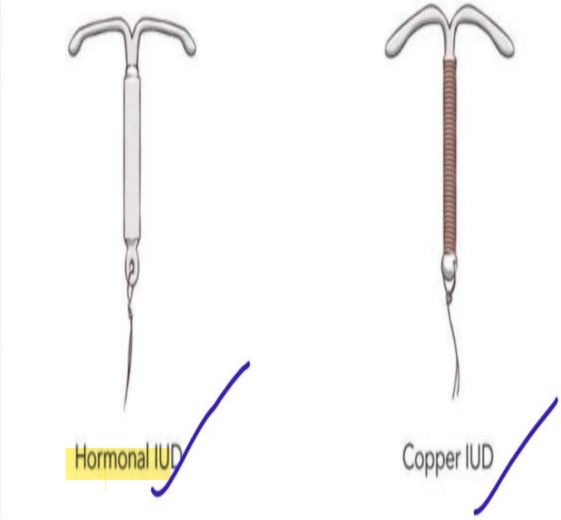
IUD (Intra-Uterine Devices)

- most widely used in INDIA
- 3 types –
  - Non-Medicated – Eg Lippes loops
    - Only phagocytosis of sperm
  - Cu^{2+} releasing – Eg Multiload-375, CuT, Cu-7



    - Suppress sperm motility & fertilizing capacity
    - Phagocytosis of sperm
  - Hormone IUDs – Eg Progestasert, LNG-20
    - Uterus – Unsuitable for implantation
    - Cervix – hostile for sperm





Hormonal Method

Usuage of hormone - Progesterone, Estrogen

Types – • Oral pills
• Implants
• Injectables



Oral Pills

Started on 3-5th day of menstrual cycle and continued for 21 days

↓

Then 7 day gap

21 days - Progesteron + Estrogen – Inhibit Ovulation

7 days – Endometrium Dreak – Periods

Implants



• Release progesterone + estrogen
• Inserted Sub-Dermally

• Inhibit ovulation & implantation

Injectables

• given in form of injection
• Release estrogen + progesterone

Surgical Methods

Vasectomy

- For males
- Stop sperm transport
- Vas-deference is cut & tied
- Incision on scrotum/abdomen

Tubectomy

- For females
- Ova transport stopped
- Fallopian tube is cut & tied
- Incision on abdomen

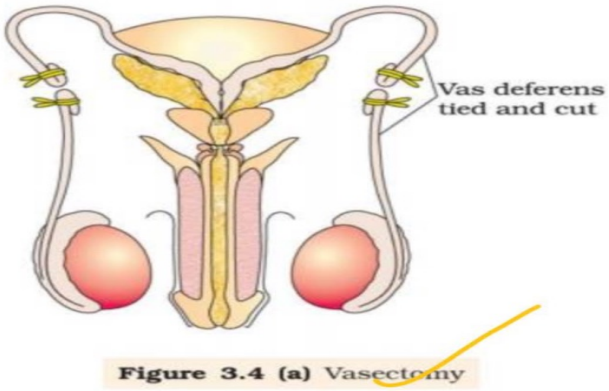


Figure 3.4 (a) Vasectomy

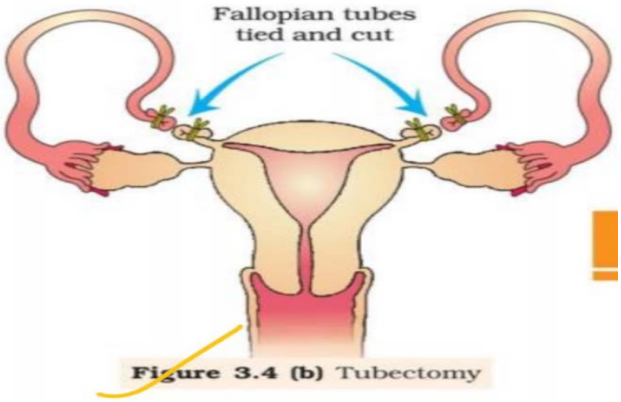


Figure 3.4 (b) Tubectomy

Emergency Methods

Within 72 hr

- pill/unwanted 72
- taken within 72 hr of having sexual intercourse

Unwanted Pregnancy

↓

MTP/Abortion

MTP (medical termination of Pregnancy)

- Used in case of unwanted pregnancy
- MTP legalized in INDIA → 1971
- Modified MTP amendment – 2017

Can be done in case of –

- Foetus has genetic abnormality
- In case of Rapes
- Case of Un-intentional pregnancy
- In case foetus cause injury to maternal organ – Ectopic Pregnancy

Time Period for MTP

Safe in 1st Trimester

↓

- Performed by registered medical Practitioner

In case of 1st-2nd trimester & 4 weeks

- Not very safe
- Can be performed – When 2 registered medical practioner gives common opinion

STD / Venereal Disease / Reproductive tract Infection

STD - Sexually Transmitted Disease

Age Group - 15-24 yrs

Early Dectection - Curable except HIV Hapes Hepatitis

Category	Disease
Bacterial	Gonorrhea, Syphilis, Chlamydiasis.
Protozoan	Trichomoniasis
Viral	Genital herpes, Hepatitis-B, Genital warts, AIDS.

HIV caused by →
- Used syringes, needles
- Mother → Placenta → foetus
- Blood transfusion

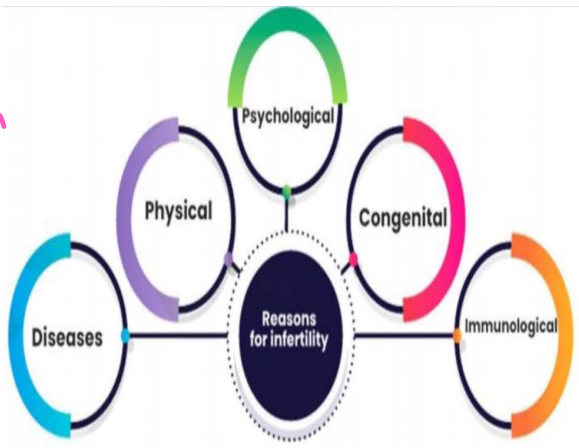
Precaution –
- Protected sexual intercourse i.e. Condom
- Avoid multiple sexual partners
- Consult a good doctor (as soon as possible)

Infertility

- Inability to produce progeny
- Unprotected sexual intercourse >2 years

Male
- lesser sperm motility
- Erectile dysfunction
- No sperm formation
- less sperm count

Female
- Fimbriae not able to catch ova
- Inable to transport ova in FT



ART (Assisted Reproductive Technology)

In Vitro Fertilization

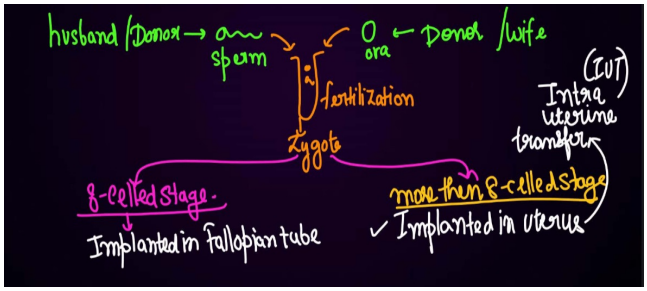
- fertilization performed outside the body
- ZIFT (Zygote intra fallopian transfer)
- ICSI (Intra-cytoplasmic sperm injection)

In Vivo fertilization

- fertilization inside the body
- GIFT (Gamete intra fallopian transfer)
- AI (Artificial insemination)

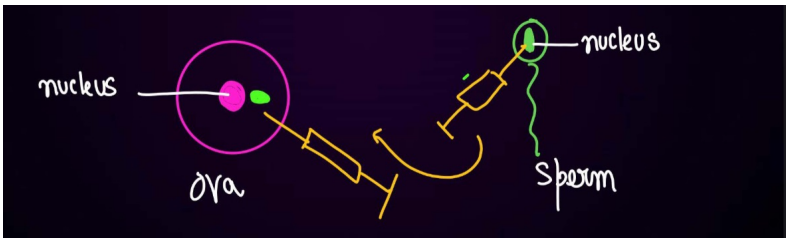
ZIFT (Zygote intra fallopian Transfer)

- Early test tube baby program



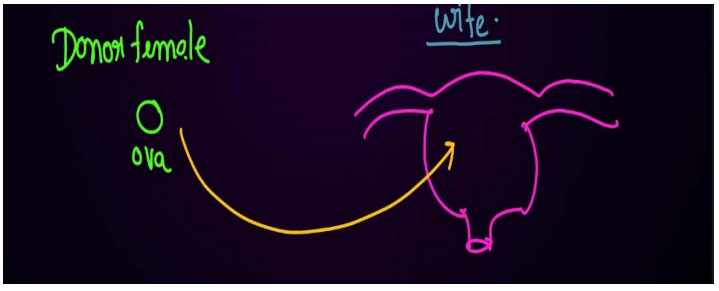
ICSI (Intra cytoplasmic Sperm Injection)

- Ova from female donor/wife is taken out and sperms nucleus is taken out via micro needle and transferred into ova



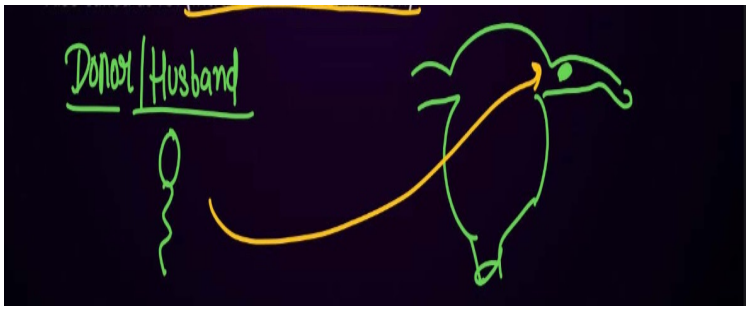
GIFT - Gamete intra fallopian transfer

Ova from female donor is taken and implanted in female/wife (infertile) body



AI (Artificial Insemination)

- Sperm is collected from husband/donor and is transferred in female uterine cavity/F.T.
- Also known as **Intra Uterine Insemination**



Adoption

- Orphaned children can be adopted by permission of law.
- Emotional, religious and social factors are also deterrments of adoption.